Capecitabine + temozolomide (capecitabine and temozolomide (CAPTEM))

Pancreatic neuroendocrine tumour (NET) oral combination This guide is for the NCCP pancreatic NET CAPTEM schedule, not a temozolomide brain-tumour or radiotherapy schedule.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

capecitabine and temozolomide (CAPTEM) · capecitabine D1–14 and temozolomide D10–14

Active medicines: Capecitabine + Temozolomide

Emetogenic risk: Capecitabine oral minimal-to-low; temozolomide oral moderate-to-high

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Capecitabine is minimal-to-low emetogenic, while temozolomide is moderate-to-high emetogenic in this oral schedule. The team must prescribe an oral anti-sickness plan for Days 10–14 and any rescue treatment; this is not a single low-risk infusion plan.
- Use the unit's mouth and skin care. Significant pain, ulcers or hand-foot symptoms may require capecitabine interruption; do not keep taking tablets just to finish the fortnight.
- Ask for written diarrhoea instructions and the point at which to stop capecitabine and call. Persistent vomiting, severe diarrhoea, dizziness or inability to drink needs urgent advice.
- The team assesses whether Pneumocystis jirovecii pneumonia (PJP) prevention is needed under local policy, especially with steroid exposure or other risk factors. CAPTEM is not the 42-day temozolomide-plus-radiotherapy schedule; do not borrow that schedule's automatic prevention plan.
- A separate prescribed oral antiemetic plan is required for temozolomide Days 10–14. The source does not specify one universal drug/dose; do not substitute a parenteral low-risk bundle.
- Day-specific administration: capecitabine with food/within 30 minutes; temozolomide fasting. Omitted toxicity doses are not replaced.
- PJP prevention is a local risk-based decision for this five-day temozolomide exposure, not automatic adoption of the 42-day brain radiotherapy regimen.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Sources checked 2026-11-05 · SACTCheck v0.84.37 · Regimen-level drafts; prescriber and oncology-pharmacy verification required